



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض رباعياً : شاهيناز محمود نورالدين علي

العنوان : العمومي

الحي / المدينة : البساتين

المحافظة : القاهرة

تاريخ تسجيل الإستمارة : 10-10-2026

رقم البطاقة : 28101108800263 رقم التليفون : 01019047204

العمر (تاريخ الميلاد) : 10-01-1981 المهنة : ربه منزل

الإقرار

أقر أنا الموقع أدناه أنني لا اتمتع بأي نظام علاج مثل :- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات أو أي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي : (.....) التوقيع: (.....)

المقرر بما فيه : (.....) التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

قسم العلاج :
الأورام

خطة العلاج : أبحاث أورام أشعه مقطعيه علي البطن والصدر والحوض
بالصبغة [11127]

التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني : الوظيفة : : التوقيع :
الاسم : د/ندى حسن : الوظيفة : : التوقيع :
الاسم : د/نرمين رمضان : الوظيفة : : التوقيع :
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

أ.د. / / د. / / د. / / د. /

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى : : قسم :
جهة العلاج : : نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

Name: شاهيناز محمود نور الدين علي
National ID: 28101108800263



تقرير طبي

المسيد

تحية طيبة وبعد,,

بشان تقرير طبي للمريض

إيماء للطلب المقدم من

رقم طبي:

بقسم:

نتشرف بان نوضح فيما يلي تقريراً عن حالته

تاريخ وساعة الخروج :
PM 01:17 05-07-2026

تاريخ وساعة الطول :
-- --:-- yyyy-mm-dd

وبعد الكشف والفحوصات والمتابعة وجد انه

المریضة تعاني من ورم غير حميد بالبدن و تحتاج الي Contrast-enhanced CT Chest & abdomen and pelvis

الوقت :

-- --:--

التاريخ :

05-07-2026

الطبيب المعالج :
د/شيماء لاشين

Name : Shahinez Mahmoud Nour El-Din Ali
Branch : Dokki
Ref. Physician Prof : Goustav Roussey Egypt

Date : 21-04-2026 07:18
Medical No. : 355685
Ins. Company : تعاونية مستشفيات

Whole Body F18- FDG PET/CT study

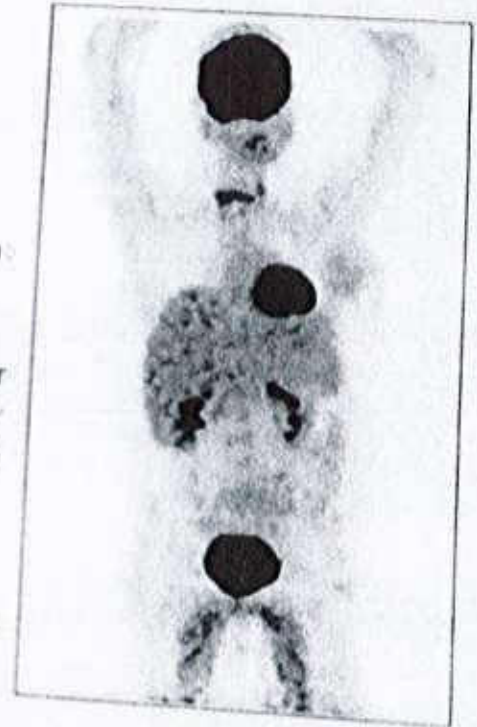
Clinical history:-

45 year old female, presenting with metastatic left breast cancer received hormonal treatment and referred for PET/CT evaluation.

Compared to the previous PET/CT dated 02/03/2025

Procedure:

- ✓ Radiopharmaceutical: 18F FDG, 4 mCi IV.
- ✓ Blood glucose level at time of 18F-FDG injection: 112mg/dL.
- ✓ Time from 18F-FDG injection to scan: 60 min
- ✓ Technique :MDCT diagnostic examination was taken for attenuation correction anatomic localization followed by PET images from the skull vault to the mid thigh were obtained.
- ✓ Images of CT and corresponding functional PET images are taken in axial, coronal and sagittal planes.
- ✓ Patient's height: 160 cm patient's weight: 85 kg
- ✓ SUV average of reference hepatic activity=2.1.
- ✓ Measurements are correlated to body weight.



PET/CT FINDINGS:-

Head and Neck:

- No metabolically active cervical or supra-clavicular LNs noted.
- The brain exhibited normal FDG bio-distribution
- Physiologic FDG uptake is seen in the oropharynx, salivary glands, and larynx.
- Normal thyroid gland FDG uptake with no focal lesions.
- Left maxillary sinusitis.

Call Center 19656

Name : Shahinez Mahmoud Nour El-Din Ali

Date : 21-04-2026 07:18

Branch : Dokki

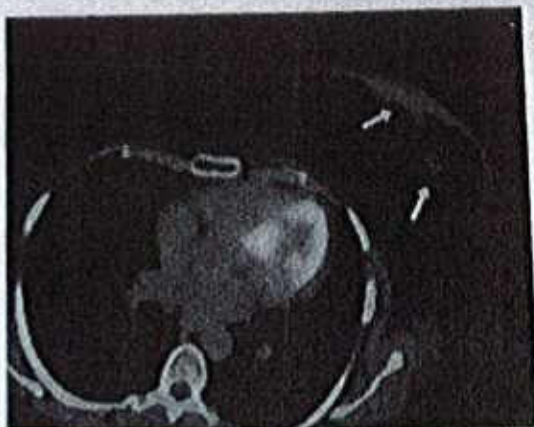
Medical No. : 355685

Ref.Physician Prof : Goustav Roussey Egypt

Ins.Company : تعاف مستشفيات

Chest:

- Further regression of the previously noted left breast lesions, currently an ill-defined non-FDG-avid lesion seen at the outer quadrant measuring 17 x 10 mm [compared to 36 x 11 mm].
- Still noted left breast diffuse skin thickening reaching maximum thickness of 24 mm achieving SUV max of 2.4 [compared to 17 mm and SUV max 1.8].
- Further morphological regression of the previously noted non-FDG-avid left axillary lymph nodes measuring up to 10 mm [compared to 17 mm].
- Still noted non-FDG-avid few tiny bilateral pulmonary nodules measuring up to 5 mm.
- No metabolically active right breast lesions.
- No metabolically active mediastinal, axillary or hilar lymph nodes noted.
- No FDG avid pulmonary nodules.
- There are no pleural or pericardial abnormalities.
- Physiologic FDG uptake is noted in the myocardium.



Abdomen and Pelvis:

- The liver shows diffuse hypoattenuation due to diffuse hepatic steatosis. No FDG avid lesions.
- No evidence of splenic FDG avid focal lesions.
- No metabolically active pelvi-abdominal LNs noted.
- There are no FDG avid adrenal nodules.
- No evidence of FDG avid peritoneal nodules.
- Physiologic FDG excretion is seen in the kidneys and bladder. Retroaortic left renal vein.
- No evidence of ascites.

Call Center 19656

Name: شاهيناز محمود نور الدين علي		
MR. : 4832		
Detailed Medical Report		
		السيد
تحية طيبة وبعد,,		
إيماء للطلب المقدم من	بشأن تقرير طبي للمريض	
بقسم:	رقم طبي:	
ننشرف بأن نوضح فيما يلي تقريراً عن حالته		
تاريخ وساعة الدخول :	تاريخ وساعة الخروج :	
وبعد الكشف والفحوصات والمتابعة وجد انه		
The patient has a newly developed osseous lesion, and the multidisciplinary team (MDT) discussed the possibility of adding a CDK4/6 inhibitor to the treatment regimen.		
The MDT concluded that the patient will receive a regimen consisting of IBRANCE, ZOLADEX, and ZOMETA administered every three months.		
Approval for FASLODEX and IBRANCE is pending; today the patient continues on FEMARA with supportive treatment planned for three months.		
The first cycle of therapy was initiated with FASLODEX, IBRANCE, ZOMETA, and ZOLADEX; laboratory results are pending.		
Laboratory results have been accepted, and the patient is scheduled to receive IBRANCE 125 mg in conjunction with FASLODEX on 7 June 2026 at the Day Care pharmacy, with ZOMETA to be administered at the next visit approximately one month later.		
The patient has a history of left-sided breast cancer, clinical stage T4b N2 M1 with bone metastases; pathology shows invasive ductal carcinoma, grade 2, estrogen receptor 8/8 positive, progesterone receptor 5/8 positive, HER2 2+ non-amplified. The patient has been on ZOLADEX AI since October 2024 until February 2026. New osseous lesions have been identified in the cervical vertebrae, prompting initiation of FASLODEX, IBRANCE, and ZOMETA for palliation of cervical vertebral involvement; mild right upper limb weakness is noted. The scheduled due date for the combined regimen of IBRANCE, FASLODEX, and ZOLADEX is 2 August.		
The current plan includes IBRANCE 125 mg to be taken daily starting 30 August 2026, and FASLODEX 500 mg to be administered on 13 September 2026, with the next follow-up visit to be scheduled thereafter.		
تحريراً في ,		
التاريخ :	19-09-2026	الوقت :
الطبيب المعالج :	Reham Ahmed Fathy Ibrahim	
يعتمد ,		

Name : Shahinez Mahmoud Nour El-Din Ali

Date : 21-04-2026 07:18

Branch : Dokki

Medical No. : 355685

Ref.Physician Prof: Goustav Roussey Egypt

Ins.Company : تعاليد مستشفى

- Still noted uterine body large fibroid measuring 104 x 98 mm. An IUCD is seen displaced to the right side.

Musculoskeletal:

- Newly developed FDG avid sclerotic osseous lesion seen implicating CV 7 vertebral body and both lamina achieving SUV max of 7.5.
- Right acetabular roof bony island.
- Left acetabular anterior column lytic osseous lesion.
- No other metabolically active lytic or sclerotic osseous lesions.
- Spondylodegenerative changes of the surveyed spine.

Conclusion:

Follow-up PET/CT study revealed:

- Further regression of the left breast primary neoplastic lesions as described above.
- Further morphological regression of the regional axillary lymph nodes.
- Newly developed metabolically active CV 7 sclerotic osseous lesion for correlation with tumor markers and follow-up.

Thanks for referral;
Dr. Heba Elnaggar, MD
Lecturer of Radio-diagnosis,
ASUH



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ربة منزل

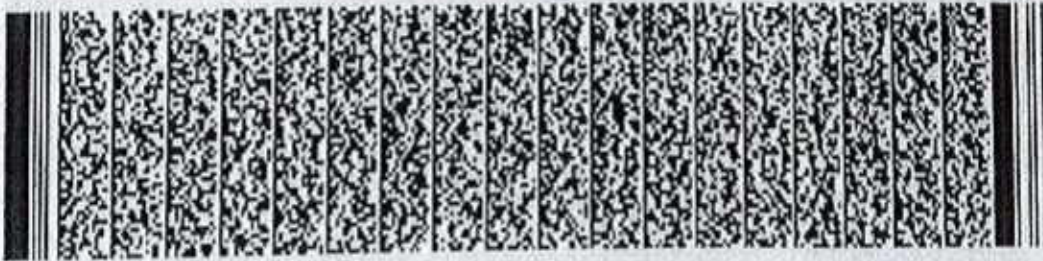


أنثى مسلمة متزوجة

امجد حسين الفولى حجازى

البطاقة سارية حتى ٢٠٢٨/١٠/١٩

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جمهورية مصر العربية

بطاقة تحقيق الشخصية

شاهيناز

محمود نور الدين على

٣٧ ش احمد زكى - العمومى

البساتين - القاهرة

١٩٨١/٠١/١٠

٢٨١٠١١٠٨٨٠٠٢٦٣

JC4086689

Name : Shahinez Mahmoud Nour El-Din Ali
Branch : Dokki

Date : 07-02-2026 10:50
Medical No. : 311283

Tc^{99m} MDP SKELETAL SCINTIGRAPHY

History

- A 43-year-old lady, with history of breast cancer and referred for follow up.

Technique

- Anterior and posterior whole-body scans were acquired 2 hours post injection of the tracer.

Findings

Considering the prior study dated 18-9-2025;

- Newly-seen active tracer uptake at few cervical, dorso lumbar vertebrae, proximal both humeri and pelvic bones.
- No other abnormal active or cold focal bony lesions that could account for metastatic bone disease.
- Degenerative changes are noted at the spine and big joints.

Impression:

- Active metastatic osseous lesions at the aforementioned sites.

Best regards,
Dr. Alaa Nagi, MD
Nuclear Medicine

**GUSTAVE/
ROUSSY**
INTERNATIONAL

shahinaz nahmoud nour al din
GRI-111200

**POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST WITH
CORONAL RECONSTRUCTION: -**

Findings:

**In comparison to previous study dated 30/8/2025 current
study reveals:**

- Left breast outer quadrant soft tissue lesion measures 29x16 mm was 26x19 mm with skin thickening measures 21 mm was 22 mm also lateral breast nodule measures 5.6 mm was 8 mm.
- Still noted right upper lobe pulmonary nodules largest at the posterior segment measures 3.5 mm was 4.9 mm.
- No hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.
- Normal appearance of the chest wall.
- Normal appearance of the dorsal vertebrae.
- Upper abdominal cuts are unremarkable.

OPINION:

Stationary course of Left breast outer quadrant soft tissue lesion also lateral breast nodule for US correlation.

Stationary course of right upper lobe pulmonary nodules could be metastatic for follow up.
Dr. Mai Seddik

**MULTI-SLICE CT SCAN OF THE ABDOMEN AND PELVIS AFTER
BOTH ORAL & I.V. CONTRAST ADMINISTRATION: -**

Findings:

- The liver is enlarged in size with low homogenous CT texture. No focal lesions or contour irregularities.
- Uterine submucosal fibroid measures 10x8.5 cm was 9.6x8.5 cm in axial cuts.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.
- Clear scanned lung bases.

Opinion:

Fatty hepatomegaly.

Stationary uterine submucosal fibroid for US correlation and MRI pelvis if clinically warranted.

Dr Mai Mohamed MD

2/22/26, 12:42 PM

code:02 -- Name_A MDT -- Name_E MDT

جوسناف
روبي
مصرمبادرة رئيس الجمهورية
لدعم صحة المرأة المصريةالهيئة العامة
لصحة
الأسرة

02/22/2026

MDT FORM

Name:	/ Mohamad Nour Al Deen Ali	MRN:	4832
Age:	45 Y	ID:	
Menopausal Sta	premenpausal	Phone:	01099754914
NRS SCORE:		Marital Status:	married
BMI:		Family History:	no
CO Morbidity:	no	Reason Of MDT:	disease progression

Clinical Data:

BX:	IDC						
ER:	8/8	PR:	5/8	HER2:	NOT AMPI	KI67:	

Sonomammography:

Mets Workup:

METASTATIC TO BONE

History:

acase of premenpausal metastatic left breast ca to bone since 2024 received AI/ZOLADEX/ ZOMETA EVERY 3 M BONE SCAN 7/2/2026.. NEWLY DEVELOPED TRACER UPTAKE AT FEW CERVICAL ,DORSOLUMBAR, AND PROXIMAL HUMERI AND PELVIC BONE

Recommendations:

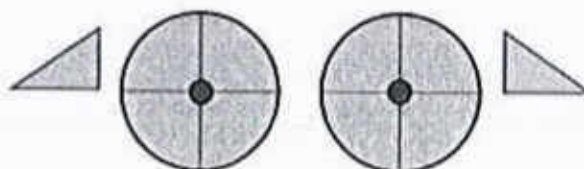
ADDING CDK4/6INHIBITOR

MDT Outcome & Plan

FOR ibrance faslodex. zoladex
zometa every 3 ms.

R

L



TNM

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د/ حنان عورت		د/ احمد نجيب			

Patient Name: Shahinaz Mahmoud Nour Al Din Ali	Date : 30/08/2025
Exam: CT CAP+C (CT Chest Abd. Pelvis)	Age : 44 Year(s)
Medical ID: 3204/2024	Pat. ID: 111200

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN AND PELVIS

Clinical History:

- Known case of left breast cancer, no available previous films for comparison.

CT SCAN OF THE CHEST

Findings:

- Left breast heterogeneous parenchyma is noted associated with overlying skin thickening reaching 13 mm.
- Few subcentimetric left axillary lymph nodes.
- Few prominent right axillary lymph nodes.
- Anterior mediastinal soft tissue density is noted measuring about 2x4 cm.
- Few scattered right lung tiny pulmonary nodules are noted average 3 mm.
- No sizable hilar or mediastinal lymph nodes.
- Patent tracheo-bronchial tree.
- Normal CT appearance of the mediastinal vessels.
- Average size of the heart.
- No evidence of pleural or pericardial effusion.



of

OPINION:

- Left breast heterogeneous parenchyma associated with overlying skin thickening.
- Few subcentimetric left axillary lymph nodes.
- Few prominent right axillary lymph nodes.
- Anterior mediastinal soft tissue density (?thymic).
- Few scattered right lung tiny pulmonary nodules.

CT SCAN OF THE ABDOMEN AND PELVIS: -



Findings:

- The liver is enlarged (21.5 cm) with diffuse low homogenous CT texture. No sizable focal lesions.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Average size of the spleen (12 cm) with no focal lesions seen.
- Normal size, parenchymal thickness and density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas and both suprarenal glands.
- Average filling of the urinary bladder with no sizable stones or masses.
- Bulky uterus showing isodense lesion for TVUS correlation. IUD is noted.
- NB. L5V mixed lytic/sclerotic lesion, left acetabulum lytic lesion and right iliac bone sclerotic lesion.

Opinion:

- *Fatty hepatomegaly for clinical and lab correlation.*
- *Bulky uterus showing isodense lesion for TVUS correlation.*
- *L5V mixed lytic/sclerotic lesion, left acetabulum lytic lesion and right iliac bone sclerotic lesion.*

Marina Essam, MSc
Dr.Noura Nabil,MD

Signature Noura Nabil
04/09/2025 2:13 PM



SCAN & LABS

مركز النيل للأشعة والتحاليل

Name : Shahinez Mahmoud Nour El-Din Ali

Date : 18-09-2025 02:31

Branch : Dokki

Medical No. : 205955

Tc^{99m} MDP SKELETAL SCINTIGRAPHY

History:

- 43-year-old lady, with history of left breast cancer, referred for follow up.

Technique:

- Anterior and posterior whole body scans were acquired 2 hours post injection of the tracer.

Findings:

- No abnormal active or cold focal bony lesions that may account for metastatic bone disease could be seen all over the surveyed skeleton.
- N.B: The prior PET/CT reported metastatic lesions at both humeri, multi-levels of the spine, sacrum, pelvic bones and both femora; are not active in our study; denoting good therapy response.
- Degenerative changes are seen at the spine and big joints.

Impression:

- The skeleton is free from active metastases.

Best regards,
Dr. Alaa Nagi, MD
Nuclear Medicine

Patient Name : الأستاذ / شاهيناز محمود نور الدين علي
Patient No. : 28101108800263
Path No. : 3038/24
Sex : Female
Age : 43 Y
Referring Doc :

Request No. : 13003586
Request Date : 21/08/2024
Verification Date : 27/08/2024
Financial Category : نفقة الدولة - اورام دار السلام
Department : Out Patient
Registered by : Mrs. Mayada Hegazy

Pathology Report

Clinical data

Left breast mass

Nature of specimen

Tru cut biopsy, collected at 21/8/2024, 10:30 am

Gross examination

Multiple grayish white tissue cores up to 1 cm, totally submitted.

Microscopic examination

- * Histological type: Invasive duct carcinoma.
- * Grade (differentiation + nuclear pleomorphism + mitosis): (3+2+2) Grade
- * DCIS: No
- * Angioinvasion: Couldn't be detected in the received biopsy
- * Perineural invasion: No

Diagnosis

Left breast mass, Tru cut biopsy:

Invasive duct carcinoma, Grade II.

For further IHC (ER, PR, HER2, Ki67)

Signature

Dr. Walaa Ghoneim

Dr. Heba Hassan

Verified by :

Printing Date : 19/09/2024 08:41:59

Page 1 of 2

Printing By : Mrs. Mayada Hegazy

CAP
ACCREDITED

غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor - Building No. 987 - Nile Corniche - Mizr El Qadema - Cairo, Egypt

Patient Name : الأستاذ / شاهيناز محمود نور الدين علي
 Patient No. : 28101108800263
 Path No. : 3038/24
 Sex : Female
 Age : 43 Y
 Referring Dcc. :

Request No. : 13003586
 Request Date : 21/08/2024
 Verification Date : 09/09/2024
 Financial Category : نفقة الدولة - اورام دار السلام
 Department : Out Patient
 IHC No. :
 Registered by : Mrs. Mayada Hegazy

IMMUNOHISTOCHEMISTRY REPORT

Clinical data:
 Left breast mass.

Nature of specimen:
 Tru cut biopsy, collected at 21/8/2024, 10:30 am.

Technique:
 - Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against:
 - ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Results:
 * IPT (Positive control showed appropriate reactivity):
 - ER: Strong nuclear reaction in 97% of tumor cells (score 8/8).
 - PR: Strong nuclear reaction in 10% of tumor cells (score 5/8).
 - Her2: Moderate complete membrane reaction in >10% of the tumor cells (score 2+).
 - Ki67: 10% nuclear reaction of tumor cells.

Conclusion:
 Left breast mass, Tru cut biopsy:
 - Invasive duct carcinoma, Grade II.

IPT:
 - ER: Positive (score 8/8).
 - PR: Positive (score 5/8).
 - Her2: Equivocal (score 2+) → not amplified.
 - Ki67: Positive in 10% of tumor cells.

Verified by :

Printing Date : 19/09/2024 08:42:00

Page 1 of 2

Printed By : Mrs. Mayada Hegazy



Patient Name	الأستاذة / شاهيناز محمود نور الدين علي	Request No.	13003586
Patient No.	28101108800263	Request Date	21/08/2024
Path No.	3038/24	Verification Date	09/09/2024
Sex	Female	Financial Category	أورام دار السلام - نفقة الدولة
Age	43 Y	Department	Out Patient
Referring Doc.			

Registered by Mrs. Mayada Hegazy

IMMUNOHISTOCHEMISTRY REPORT

* Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak; 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature

Dr. Walaa Ghonaim.

Dr. Toka Medhat.

Verified by :

Printing Date : 19/09/2024 08:42:00

Page 2 of 2

Printed By : Mrs. Mayada Hegazy

Patient Name : الأستاذ / شاهيناز محمود نور الدين علي

Request No. : 13003586

Patient No. : 28101108800263

Request Date : 21/08/2024

Path No. : 3038/24

Verification Date : 16/09/2024

Sex : Female

Financial Category : نفقة الدولة - اورام دار السلام

Age : 43 Y

Department : Out Patient

Referring Doc :

Registered by : Mrs. Mayada Hegazy

Pathology Report

Clinical Data: Left breast mass.

Nature of Specimen: Tru-cut biopsy, collected at 21/8/2024, 10:30 am.

Technique:

- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against Her2 clone (4B5) & SISH.

Results:

* IPT (Positive control showed appropriate reactivity):

- Her2: Moderate and complete membrane reaction in >10% of tumor cells (score 2+).

- SISH: Not Amplified, group V.

Average Her2 copy No. > (2.95), Her2/CEP17 ratio > (1.78).

Conclusion:

Left breast mass, Tru-cut biopsy:

Invasive duct carcinoma, Grade II.

IPT:

- Her2: Equivocal (score 2+).

- SISH: Not Amplified, group V.

Signature

Dr. Walaa Ghonaim.

Dr. Toka Medhat.

Verified by :

Printing Date : 19/09/2024 08:41:58

Page 1 of 2

Printing By : Mrs. Mayada Hegazy

CAP
ACCREDITED

غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[DOCTYPE:PATHOLOGY_REPORT]



الهيئة العامة للمستشفيات والمعاهد التعليمية
مستشفى أحمد ماهر التعليمي

The General Organization for Teaching Hospitals
and Institutes

Ahmad Maher Teaching Hospital

To whom it may concern

Mrs. / Mr. شاهيناز محمد علي , 45. years old
female/ male patient presented with diagnosis Met: Breast
for to lung and Bone

She/ He was referred to us for (radical / adjuvant / Neoadjuvant /
palliative) Radiotherapy

She / He has received (3D/ IMRT) radiotherapy to C5-T10

At Dose : 20.Gy/ 5 .fx/ 1 .w .

Additional (Boost) : yes/ no. at doseGy/fx/w
(sequential / concomitant)

Started: 5/3/2020

Ended: 10/9/2020 With good tolerance .

Head of department
Dr. Hany William

د. هاني ويليام